



Barcode No 87139614

Patient Name Mrs.RIMPY

Age/Sex 37 YRS/Female

Referred By SELF

Lab No

16952407140004

Reg Date

14/Jul/2024 01:34PM

Sample Coll. Date

14/Jul/2024 12:23 PM

Sample Rec.Date

15/Jul/2024 08:07 AM

Report Date

15/Jul/2024 08:41AM

HAEMATOLOGY

Test Name With Methodology

Result

Unit

Biological Ref.Interval

Haemoglobin HPLC

Hb F(Foetal Hb) Level

0.9

%

< 2.0

Peak 2

5.2

%

Peak 3

3.3

%

<10

Hb A0

82.2

%

HbA2

3.1

%

1.9 - 3.5

Hb D (Punjab)

0.0

%

<0.02

HPLC

Hb Sickle

0.0

%

<0.02

Hb C

0.0

%

<0.02

HPLC

Hb E

0.0

%

<0.02

Opinion

Normal Pattern

Advise

Please Correlate Clinically

Comment:

Haemoglobinopathy is a kind of genetic defect that results in abnormal structure of one of the globin chains of the hemoglobin molecule. Hemoglobin variants are abnormal forms of hemoglobin. All results have to be correlated with age and history of blood transfusion. If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended. **In case of hemoglobinopathy, parents or family studies, DNA Analysis and Genetic counseling is advised.**

- ~ **P3 window-** Above 10% is often indicative of either denatured forms of hemoglobin (degenerated sample) or may suggest a possibility of abnormal hemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
 - ~ **P2 window-** Above 10% is indicative of either glycated hemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal hemoglobin variant requiring further DNA studies for confirmation.
 - ~ Screening of the spouse are strongly recommended, if pregnant woman have hemoglobinopathy.
 - ~ This test detects Beta thalassemia and hemoglobinopathies. DNA analysis is recommended to rule out alpha thalassemia and silent carriers.
 - ~ Mild to moderate increase in **fetal hemoglobin (HbF)** can be seen in some acquired conditions like Pregnancy, Megaloblastic anemia, Thyrotoxicosis, Hypoxic conditions, Chronic kidney disease, Recovering marrow, MDS, Aplastic anemia, PNH, Medications (Hydroxyurea, Erythropoietin, Butyrates) etc.
 - ~ Iron deficiency Anemia may be associated with **low HbA2** result and may mask thalassemia condition also.
 - ~ **Elevated levels of HbA2** are seen in Beta-Thalassemia, and also in other conditions like Sickle cell Alpha-Thalassemia, Megaloblastic Anemia, Unstable Hemoglobinopathies, Zidovudine- treated HIV patients, Congenital Dyserythropoietic Anemia (CDA) type 1 and Hyperthyroidism.
- *Results relate only to the sample, as received.

Prashant

Dr Prashant Goyal
(Chief Pathologist)

Page 1 of 1

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Chromatogram Report

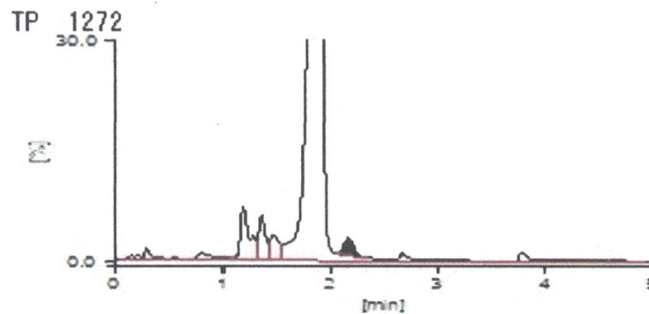
Tosoh HLC723G11 V3.07 1 2024-07-15 08:31:04
ID 87139614
Sample No. 2024071508090012 SL 0001 - 03
Patient ID
Name
Comment

CALIB F $Y = 1.2098X + 0.5211$
A2 $Y = 1.3208X + 0.8120$

Name	%	Time	Area
F	0.9	0.55	3.41
A0	82.2	1.89	995.93
A2	3.1	2.17	21.69
E+			
D+			
S+			
C+			

Total Area 1212.13

HbF 0.9 % HbA2 3.1 %



[Unknown Peak]

Name	%	Time	Area
P00	1.6	0.29	19.09
P01	0.0	0.66	0.22
P02	1.4	0.81	17.26
P03	5.2	1.20	62.70
P04	3.3	1.37	40.15
P05	2.5	1.47	30.77
P07	1.0	3.80	12.27

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